



Patient: POLLY POCKET
Date of Birth: 01/01/1929
Date: 06/13/2019 11:15 AM
Historian: Self
Visit Type: Office Visit

90 year old female presents with multiple medical conditions.

History of Present Illness:

Gastrointestinal hemorrhage with melena: She noticed blood in her stools for the past two nights. She vomited last night. Her stools are darker. No chest pain or abdominal pain. She was admitted in April 2019 in view of GI bleeding and anemia; her hemoglobin was around 6 and therefore received two units of blood. On discharge, her hemoglobin was around 8. Followed up with me after discharge and repeat blood work on April 17, 2019 revealed hemoglobin at 8.9, it was at 8.8 on May 07, 2019. She was followed up by gastroenterologist, Dr. Hillman who advised GI workup; however, she declined.

The blood work done on June 09, 2019 revealed hemoglobin at 7.5. Her daughter spoke to Dr. Hillman and is waiting for a call. She was also followed up by gastroenterologist, Dr. Geders.

NP, Karyn Murray called her as soon as she got abnormal results over the weekend.

Anemia: Takes Iron supplements two times daily. Reports weakness without any breathing issues since last night, her daughter stayed with her. Her hemoglobin "blood" is low and cold.

Hypotension: Reports tiredness and felt weak today while on the way to the office. Blood pressure as measured by the nurse today is 98/60 mmHg. Her last blood pressure was at 130/80 mmHg.

Nonrheumatic aortic valve stenosis: Has a history of severe aortic stenosis.

Review of Systems:

System	Neg/Pos	Details
Constitutional	Positive	Generalized weakness, Tiredness.
GI	Positive	Blood in stool, Vomiting, Dark stools.
Cardio	Negative	Chest pain and chest pressure.
Eyes	Negative	Vision changes.
GI	Negative	Abdominal pain.
Respiratory	Negative	Dyspnea.

Problem List:

Problem Description	Onset Date	Chronic
Hyperthyroidism	12/23/2014	
Hypertension, Unspecified	04/16/2013	Y

Mild cognitive impairment
Hypercholesterolemia

08/01/2014
04/16/2013 Y

Family History:

Relation	Name	Deceased	Death Age	Condition	Onset Age	COD
Father				Heart disease		
Father				No history of Cancer, skin		N
Mother				CVA (Stroke)		

Allergies: Reviewed, no changes.

Ingredient	Reaction	Medication Name	Comment
SULFA (SULFONAMIDE ANTIBIOTICS)			
CODEINE		Codeine	severity:

Medications:

Medication	Sig	Start Date
AMLODIPINE 5 MG TABLET	take 1 tablet by oral route every day	06/13/2019
Aspir-81 81 mg tablet, delayed release	take 1 tablet by oral route every day	
atorvastatin 20 mg tablet	take 1 tablet by oral route every day	02/23/2018
ciclopirox 8 % topical solution	apply by topical route every day to the affected area(s) preferably at bedtime or 8 hours before washing	07/20/2017
DIOVAN 80 MG TABLET	take 1 tablet by oral route every day	06/13/2019
donepezil 10 mg tablet	take 1 tablet by oral route every day in the evening	01/03/2018
ferrous sulfate 324 mg (65 mg iron) tablet, delayed release	take one tab PO QD	04/12/2018
Namenda 10 mg tablet	take 1 tablet by oral route every day	05/11/2018
pantoprazole 40 mg tablet, delayed release	take 1 tablet by oral route every day	05/11/2018
propylthiouracil 50 mg tablet	take 1 tablet by oral route 2 times every day	02/09/2018

Social History:

Never smoker.
Preferred language is English.

Alcohol:

Alcohol Use: Never.

Vital Signs:

Time	BP mm/Hg	Pulse /min	Resp /min	Temp F	Ht in	Ht cm	Wt lb	Wt oz	Hd Circ cm	BMI kg/m2	BMI %	BSA m2	O2 Sat%
11:17 AM	98/60	76		97.1	61.00	154.94	153.00			28.91	0		92

Measured By

Time	Measured by
11:17 AM	Delonda Winston

Physical Exam:

Exam	Findings	Details
Constitutional	Positive	Overall appearance - Pale and weak.
Respiratory	Normal	Inspection - Normal. Auscultation - Normal.

Cardiovascular	Positive	Heart murmur noted.
Abdomen	Normal	Inspection – Normal. Auscultation – Normal. No abdominal tenderness. No hepatosplenomegaly.
Rectal	Positive	Non-bleeding large external hemorrhoids noted. Guaiac test: Positive. Dark stool noted
Psychiatric	Normal	Orientation - Oriented to time, place, person & situation. Appropriate mood and affect.

Assessment/Plan

#	Detail Type	Description
1.	Assessment Provider Plan	Gastrointestinal hemorrhage with melena (K92.1). Guidelines for blood transfusion reviewed. Counselled on the current condition. Referral to hospital provided; will need blood transfusion, GI evaluation and other workup including endoscopy and colonoscopy to rule out the source of bleeding. I called to the ER to give report on her. Follow up with me after hospital discharge.
2.	Assessment Provider Plan	Anemia, unspecified (D64.9). Continue Iron supplements.
3.	Assessment Provider Plan	Hypotension, unspecified hypotension type (I95.9). She is hypotensive today. May be due to GI bleed. Will need to have this closely monitored in the hospital.
4.	Assessment Provider Plan	Nonrheumatic aortic valve stenosis (I35.0). Stable. May be followed up by cardiologist, Dr. Keating at the hospital.

Permission to use a Virtual Scribe was obtained during the encounter by the Nurse.

Jenny So MD
CareMount Medical
1825 Commerce Street
Yorktown Heights, NY 10598-4432
(914)962-5060

I, Bhagyashri Gitte, am serving as a scribe to document services personally performed by Jenny So MD.

Bhagyashri Gitte, 06/14/2019 05:40 AM

Document generated by: Bhagyashri Gitte 06/14/2019 05:40 AM

I personally performed the services described in this documentation, as scribed by Bhagyashri Gitte, and the documentation accurately describes the information provided by the patient and the services I performed. Any difference or additional information is noted.

Electronically signed by Jenny So MD on 06/24/2019 12:15 AM

MOUNT KISCO OFFICE
POLLY POCKET

90 SOUTH BEDFORD ROAD MOUNT KISCO, NY 10549-3412 (914)241-1050
DOB: 1/1/1929 CareMount PID#: 5203016

NORTHERN WESTCHESTER HOSPITAL

400 East Main Street
Mount Kisco, NY 10549
914-666-1200

PATIENT NAME:	POCKET, POLLY	DATE OF
BIRTH:	01/01/1929	
MEDICAL REC#:	MN00880481	ACCOUNT#:
	FN0005842853	
LOCATION:	N7S N735-A	
	06/13/19	
ADMIT/REG DATE:		DISCHARGE DATE:
REPORT DATE/TIME: 06/19/19		06/19/19

Discharge Summary
Date of Admission
Jun 13, 2019 at 16:13 Date of
Discharge
Jun 19, 2019 at 15:25
Primary Care Physician
So, Jenny MD
Consultant

Hospital Course

90 yo F, PMHx listed below sent to ED by PCP for weakness and dark stools over pat 2 days. Pt has hx of GI bleed- recently admitted for same- but deferred colonoscopy. Pt had Hgb drawn on Saturday- reportedly 7.5 and pt was in process of arranging outpt GI eval when pt began feeling generalized weakness and light headedness x 2 days and episode of vomiting x 1 last night. Pt did admit to noticing BRB in stool as well. In ED- pt noted to be anemic and while awaiting blood transfusion had acute episode of bradycardia, unresponsiveness and hypotension with SBP=79 mmHg. Patient improved with IV fluids and given blood transfusion.

Dr. Chinitz consulted and did an upper and lower scope and found no evidence of bleeding. Patient also consulted by Dr. Keating who feels the bleeding could be caused by telangiectasia for aortic stenosis and would benefit from a TVAR.

Patient medically stable for discharge with close outpatient follow-up.
Condition at Discharge
Stable

Continued Medications:
Amlodipine Besylate* (Amlodipine Besylate*) 5 Mg Tablet
5 MG PO QAM, TABLET (This prescription has been renewed)

Atorvastatin* (Lipitor*) 20 Mg Tablet
20 MG PO HS, TAB (This prescription has been renewed)

Donepezil Hcl* (Aricept*) 10 Mg Tablet

10 MG PO HS, TAB (This prescription has been renewed)

Ferrous Sulfate* (Ferrous Sulfate*) 324 Mg (65 Mg Iron) Tablet.dr
324 MG PO QBREAKFAST, #30 TABLET

Memantine HCl (Memantine HCl) 10 Mg Tablet
10 MG PO DAILY (This prescription has been renewed)

Pantoprazole Sodium* (Protonix*) 40 Mg Tablet.dr
40 MG PO QAM, #30 TAB

Propylthiouracil (Ptu) 50 Mg Tablet
50 MG PO BID, TAB (This prescription has been renewed)

Valsartan* (Valsartan*) 80 Mg Tablet
80 MG PO QAM, TABLET (This prescription has been renewed)

Discontinued Medications:

Aspirin Enteric Coated (Low Dose)* (Aspirin Enteric Coated (Low Dose)*) 81 Mg Tablet
81 MG PO QBREAKFAST, TABLET

Key Followup points

Follow-up with Dr. So on Jun 22, 2019 - at 2:30pm

Follow-up with Dr. Chinitz for capsule study.

Follow-up with Dr. Keating in 1-2 weeks, call office to schedule appointment
Problems:

(1) Gastrointestinal bleeding

Status: Acute

Summary: - s/p c-sope, no source of bleeding found

- ?telangiectasia from AS

- will need outpt capsule study w Dr Chinitz

- Hgb 9.3, Hct 28.7 on discharge

(2) Severe aortic stenosis

Status: Chronic

Summary: - has declined TAVR in the passed

- Follow-up with Cardiology as outpatient

(3) HTN (hypertension)

Status: Chronic

Summary: - monitor

(4) Hyperthyroidism

Status: Chronic

Summary: - PTU

Parkin, Amy NP

Jun 19, 2019 18:12

<Electronically signed by Amy Parkin NP>
06/19/19 1813



Patient: PDLLY PDCKET
Date of Birth: 01/01/1929
Date: 07/19/2019 03:45 PM
Historian: Self + Daughter
Visit Type: Adult Physical

90 year old female presents for a physical examination.

History of Present Illness:

She is generally feeling well. Her last physical examination was on July 11, 2018. Recently followed up with me in June 2019 in view of hospitalizations. She is accompanied by her daughter today and her other daughter is on a telephonic conversation with us for this visit.

Weight concern: Reports weight gain. Currently, weighs 150 lbs and her BMI is 28.3.

Existing medical conditions are:

Aortic stenosis: Has severe aortic stenosis. Has declined aortic valve replacement. Follows up with cardiologist, Dr. Keating and has an appointment scheduled on July 23, 2019. Had an echocardiogram done in February 2019.

She always feels short of breath on having chest pain which is not experienced since the past few weeks.

Hypertension: On Amlodipine 5 mg and Valsartan 80 mg. Blood pressure as measured by the nurse today is 150/80 mmHg.

Hypercholesterolemia: On Atorvastatin 20 mg. Mentions that blood work done in July 2018 revealed better total cholesterol levels at 136 mg/dL and LDL at 59 mg/dL. Blood work done in July 2019 revealed elevated total cholesterol levels at 260 mg/dL, elevated LDL at 174 mg/dL and elevated triglycerides at 175 mg/dL.

Hyperthyroidism: Takes Propylthiouracil 50 mg as prescribed by endocrinologist, Dr. Kleinbaum; she recently followed up with Dr. Kleinbaum. Recent blood work revealed under active thyroid function and elevated TSH levels.

Anemia: Takes Iron supplements two times daily without any abdominal issues; however, her stools are dark. She also takes vitamin C supplements as advised by me to add on the absorption of Iron. She was hospitalized for the first time in April 2019 in view of bleeding and her hemoglobin was at 6.5; received transfusion. She declined GI evaluation. She was re-hospitalized in June 2019 in view of severe anemia followed by a complete GI evaluation. Had a colonoscopy done at a hospital with gastroenterologist, Dr. Chinitz that revealed diverticulosis, a single non-bleeding angiectasia (not a cause of her anemia) and one tubular adenoma. The capsule-endoscopy was normal. She had an upper endoscopy and another digital endoscopy that did not reveal any bleeding source.

Lab work from July 14, 2019 revealed hemoglobin at 11.5 (highest level in a while).

Dr. Chinitz has advised to avoid baby Aspirin and to monitor the hemoglobin levels every 2 months going forward.

Mild cognitive impairment: Takes Namenda 10 mg and Donepezil (Aricept) 10 mg once daily since the last 6 years. Follows up with neurologist, Dr. Suzanne Brown.

Daily activities: Lives on her own and her daughters help in between. Does not drive or clean; however, cooks occasionally. Uses a life-line and goes shopping with her daughters. Has minimal balance issues; however, no recent falls. Uses a quad cane and can climb up 4-5 stairs. Goes to the senior center on Mondays.

Lab Work: Recent blood work revealed normal CBC, electrolytes, proteins, kidney function, liver function and elevated blood glucose levels at 105. Urinalysis revealed normal findings.

Immunization status:

Receives annual flu shots.

Received Tdap in July 2006. Up-to-date with Tetanus vaccine in April 2014.

Received old shingles vaccine in October 2009. Experienced mild flu symptoms. Inquires if she can get the Shingrix vaccine with flu shot.

Received Pneumovax in the past and Prevnar in March 2015.

Health maintenance:

Gynecology: Does not follow up with a gynecologist in view of her age.

Mammogram: Does not get mammogram done in view of her age.

Ophthalmology: Due. Has an appointment scheduled with ophthalmologist, Dr. Cummins in October 2018. Wears distance glasses for watching T.V.

Dermatology: Up-to-date. Followed up with dermatologist, PA Courtney Emerson in May 2019 for a complete skin check up.

Other medications: Takes Pantoprazole, vitamin E, Centrum (multivitamin) and calcium supplements.

Additional Comments: Wears hearing aids in one ear and mentions that the battery is not working. Her healthcare proxies are her daughters.

Review of Systems:

System	Neg/Pos	Details
GI	Positive	Dark stools.
Neuro	Positive	Memory impairment.
ENMT	Positive	Wears hearing aids, Wears distance glasses.
Constitutional	Positive	Weight gain.
Neuro	Negative	Dizziness, headache and frequent falls.
MS	Negative	Joint pain.
Psych	Negative	Anxiety, feeling down, depressed or hopeless and little interest or pleasure in doing things.
Cardio	Negative	Chest pain.
Respiratory	Negative	Dyspnea.
Integumentary	Negative	Hives.
GU	Negative	Urinary issues.
GI	Negative	Abdominal pain, blood in stool, change in bowel habits, constipation and black stools.

Problem List:

Problem Description	Onset Date	Chronic
Hyperthyroidism	12/23/2014	
Hypertension, Unspecified	04/16/2013	Y
Mild cognitive impairment	08/01/2014	
Hypercholesterolemia	04/16/2013	Y

Past Medical/Surgical History:

Disease/disorder	Onset	Management	Date
		status post left hip replacement in 2009, right hip replacement in 2010, tonsillectomy, D and C	

hypertension, hypercholesterolemia,
hyperthyroidism, cognitive impairment,
severe aortic stenosis, and osteoarthritis

Family History:

Relation	Name	Deceased	Death Age	Condition	Onset Age	COD
Brother		Y	82	found dead at home, unknown cause.		N
Brother		Y	41			
Brother		Y	41	MI		N
Brother				Abdominal aortic aneurysm		N
Father		Y	51	Heart disease		
Father				No history of Cancer, skin		N
Mother		Y	71	CVA (Stroke)		

Allergies: Reviewed, no changes.

Ingredient	Reaction	Medication Name	Comment
SULFA (SULFONAMIDE ANTIBIOTICS)			
CODEINE		Codeine	severity:

Medications:

Medication	Sig	Start Date
AMLODIPINE 5 MG TABLET	take 1 tablet by oral route every day	05/18/2019
atorvastatin 20 mg tablet	take 1 tablet by oral route every day	02/23/2018
DIOVAN 80 MG TABLET	take 1 tablet by oral route every day	05/18/2019
donepezil 10 mg tablet	take 1 tablet by oral route every day in the evening	07/09/2018
ferrous sulfate 324 mg (65 mg iron) tablet, delayed release	take one tab PO QD	07/09/2018
Namenda 10 mg tablet	take 1 tablet by oral route every day	05/11/2019
pantoprazole 40 mg tablet, delayed release	take 1 tablet by oral route every day	05/11/2019
propylthiouracil 50 mg tablet	take 1 tablet by oral route 2 times every day	02/09/2018

Social History:

Never smoker.

Preferred language is English.

Alcohol:

Alcohol Use: Never.

Vital Signs:

Time	BP mm/Hg	Pulse /min	Resp /min	Temp F	Ht in	Ht cm	Wt lb	Wt oz	Hd Circ cm	BMI kg/m2	BMI %	BSA m2	O2 Sat%
3:59 PM	150/80	82		98.0	61.00	154.94	150.00			28.34	0		

Measured By

Time Measured by
3:59 PM Delonda Winston

Physical Exam:

Exam	Findings	Details
Constitutional	Positive	Overall appearance - Pale and weak.
Eyes	Normal	Conjunctiva - Right: Normal, Left: Normal. Pupil - Right: Normal, Left: Normal. Fundus - Right: Normal, Left: Normal.
Ears	Positive	Minimal cerumen impaction noted bilaterally. A red spot noted on the left ear.
Nasopharynx	Positive	Teeth - bridge lower noted.
Neck Exam	Normal	Oropharynx - Normal.
Lymph Detail	Normal	Inspection - Normal. Palpation - Normal. Thyroid gland - Normal. Range of motion - Normal.
Breast	Normal	No cervical or supraclavicular adenopathy.
Respiratory	Normal	Inspection - Normal. Auscultation - Normal.
Cardiovascular	Positive	A systolic murmur noted.
Vascular	Normal	Heart rate - Regular rate. Rhythm - Regular.
Abdomen	Normal	Pulses - Dorsalis pedis: Normal. Bruits - Carotids: Absent. No JVD.
Skin	Normal	Inspection - Normal.
Musculoskeletal	Positive	Feet: Bony deformities noted. Left knee: Swelling noted.
Extremity	Positive	Feet: Swelling (L>R) noted up to the shin. Varicose veins (L>R) noted.
Neurological	Positive	Mini-cognitive test: Abnormal (2/5).
Psychiatric	Normal	Cranial nerves - Cranial nerves I grossly intact. Appropriate mood and affect.

PE Summary:

Blood pressure as measured by Dr. Jenny So today is 130/70 mmHg.

Assessment/Plan

#	Detail Type	Description
1.	Assessment	Encounter for general adult medical examination without abnormal findings (Z00.00).
	Provider Plan	The physical examination is normal except for the appearance, cerumen, heart murmur, lower extremity swelling and the bony deformities. Mini-cognitive test performed today is abnormal; the score is 2 out of 5. Reviewed blood work. Maintain healthy diet. Encouraged to continue regular exercise and stay physically active. Strongly recommended to avoid taking vitamin E supplements unless required.

Immunization status:

Up-to-date with flu shot, tetanus, old shingles and both the pneumonia vaccines.
Recommended Shingrix vaccine. Explained in details.

Health maintenance:

Colonoscopy: Up-to-date.

Ophthalmology: Due. Follow up with ophthalmologist, Dr. Cummins in October 2018.

Dermatology: up-to-date.

Follow up in 3 months or early as needed.

2. **Assessment:** Aortic stenosis, severe (I35.0).
 Provider Plan: Discussed aortic valve replacement.
 Follow up with cardiologist, Dr. Keating; may advise to restart baby Aspirin as her GI evaluation including the bleeding source is negative.

3. **Assessment:** Essential (primary) hypertension (I10).
 Provider Plan: Well controlled.
 Continue Amlodipine 5 mg and Diovan 80 mg.
 Discussed Valsartan and its brand issues.

 Advised to keep her legs in an elevated position in view of lower extremity swelling.
 Recommended to keep a check on salt and sodium intake.

4. **Assessment:** Pure hypercholesterolemia, unspecified (E78.00).
 Provider Plan: Reviewed blood work; elevated.
 Continue Atorvastatin 20 mg.
 Follow up with cardiologist, Dr. Keating.

5. **Assessment:** Hyperthyroidism (E05.90).
 Provider Plan: Reviewed blood work.
 Continue Propylthiouracil 50 mg.
 Follow up with endocrinologist, Dr. Kleinbaum.

6. **Assessment:** Anemia, unspecified (D64.9).
 Provider Plan: Reviewed blood work.
 Continue Iron supplements two times daily along with vitamin C.
 Repeat the blood work for CBC every month.
 Based on the lab results, the medications may be discontinued in near future.

7. **Assessment:** Mild cognitive impairment, so stated (G31.84).
 Provider Plan: Continue Namenda 10 mg and Donepezil (Aricept) 10 mg.
 Discussed on discontinuing the medication in detail.
 Follow up with neurologist, Dr. Brown as needed.

Additional plan details:**Elevated blood glucose levels:**

Keep a check on sweet intake.

Arthritis: Stay active.

Permission to use a Virtual Scribe was obtained during the encounter by the Nurse.

Jenny So MD
CareMount Medical
1825 Commerce Street
Yorktown Heights, NY 10598-4432
(914)962-5060

I, Bhagyashri Gitte, am serving as a scribe to document services personally performed by Jenny So MD.

Bhagyashri Gitte, 07/20/2019 02:18 AM

Document generated by: Bhagyashri Gitte 07/20/2019 02:18 AM

I personally performed the services described in this documentation, as scribed by Bhagyashri Gitte, and the documentation accurately describes the information provided by the patient and the services I performed. Any difference or additional information is noted.

Electronically signed by Jenny So MD on 08/10/2019 09:56 AM

ADDENDUMS APPEAR ON FOLLOWING PAGE